

ABDOMINAL PAIN

Give urgent attention to the patient with abdominal pain and any of:

- Chest pain →37
- Pregnant →159
- Recent delivery/miscarriage/termination of pregnancy →164
- Glucose $\geq 11.1 \rightarrow 17$
- Unable to pass urine →59
- Jaundice
- Abdominal or pelvic mass
- Pulsatile abdominal mass: **abdominal aortic aneurysm** likely

- Pain in right lower abdomen with nausea/vomiting/fever: **appendicitis** likely
- Guarding, rigidity or rebound tenderness: **peritonitis** likely
- Severe pain in right upper abdomen with nausea/fever/loss of appetite: **cholecystitis** likely
- Sudden severe upper abdominal pain spreading to back with nausea/vomiting: **pancreatitis** likely
- No stools or flatus/wind for past 24 hours
- If sudden abdominal pain and any of: generalised itch/rash, face/tongue swelling, difficulty breathing, BP < 90/60, dizziness/collapse or exposure to possible allergen¹ check for anaphylaxis → 20.

Manage and refer urgently:

- If **abdominal aortic aneurysm** likely: avoid giving IV fluids even if BP < 90/60 (raising blood pressure may worsen rupture).
- If BP < 90/60 or **pancreatitis** likely, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- If pain severe, give **morphine** 10mg IM or diluted **morphine**² 3-10mg slow IV: start with 3mL IV over 3 minutes. If needed, give another 1mL/minute until pain improved, up to 10mL. Stop if BP drops < 90/60.

Approach to the patient with abdominal pain not needing urgent attention:

- If cramping abdominal pain with recent onset vomiting, diarrhoea, loss of appetite, body pain or fever, **gastroenteritis** likely →45.
- If on ART, check for urgent side effects → 116.
- If urinary symptoms (burning/frequency/urgency) or leucocytes/nitrites/blood on dipstick →59.
- Is pain in the lower abdomen *and* is patient a woman?

Yes

- Exclude pregnancy → 157. If pregnant, refer urgently same day.
- If crampy lower abdominal pain only during periods, **dysmenorrhoea** likely →56.
- Ask about abnormal vaginal discharge and do pelvic examination to check for pain on moving cervix:

Abnormal vaginal discharge or pain on moving the cervix

Treat for **lower abdominal pain (LAP)** syndrome:

- If temperature $\geq 38^{\circ}\text{C}$, pulse > 100 or BP < 90/60: give IV fluids as above, **ceftriaxone** 1g IV³/IM and **metronidazole**⁴ 400mg orally and refer same day.
- Assess and advise patient → 49.
- Give single dose **ceftriaxone** 250mg IM⁵ and **azithromycin** 1g and **metronidazole**⁴ 400mg 12 hourly for 7 days. If severe penicillin allergy⁶, omit ceftriaxone and increase **azithromycin** dose to 2g.
- For pain, give **ibuprofen**⁷ 400mg 8 hourly with food for up to 5 days.
- Give partner notification slip/s with code: LAP.
- Advise to return if no better within 3 days or urgently if worse: refer. Otherwise, review in 7 days.

No abnormal discharge *and* no pain on moving the cervix

- If weight loss → 23.
- If recurrent pain/discomfort and ≥ 2 of: pain relieved with passing stool, abdominal distension, change in stool frequency/appearance, mucous in stool, **irritable bowel syndrome (IBS)** likely. Refer to doctor to confirm diagnosis and dietician for dietary advice.
- If constipated →48. If diarrhoea →46.

No

Does patient have epigastric pain which is worse with eating, hunger or lying down/bending forward?

No

Yes

Dyspepsia (heartburn) likely

- Advise to stop NSAIDs (e.g. ibuprofen/aspirin), stop smoking → 141, limit alcohol, caffeine, spicy food, fizzy drinks, late night meals.
- In past year, has patient: 1) drunk ≥ 4 drinks⁸/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142.
- If waist circumference > 80cm (woman) or 94cm (man), assess CVD risk → 127.
- Give **lansoprazole**⁹ 30mg daily for 14 days.
- Refer same week if any of: Hb < 12 (woman) or < 13 (man), new pain and > 50 years, or family history of stomach/oesophageal cancer.
- Advise to return if: no better after 7 days, symptoms return, difficulty swallowing, persistent vomiting, blood in vomit or stool, weight loss. Refer.

If no better or diagnosis uncertain, discuss/refer.

¹Common allergens include medication, food or insect bite/sting within the past few hours. ²Dilute 10mg morphine with 9mL of sodium chloride 0.9%. ³Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ⁴Advise no alcohol until 24 hours after last dose of metronidazole. ⁵For ceftriaxone 250mg IM injection: dissolve 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ⁶History of anaphylaxis, urticaria or angioedema. ⁷Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease. ⁸One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ⁹If HIV positive on atazanavir/ritonavir, avoid lansoprazole, discuss/refer.